

Section 8

Federal Intervention & Judicial History

Version 2.0 — Preliminary Draft

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SECTION 8 — FEDERAL INTERVENTION & JUDICIAL HISTORY

(Escalation & Enforcement Layer — "What happens when federal oversight falls short")

Version 2.3 — June 5, 2026

8.1 PURPOSE AND SCOPE

This section documents the history of federal enforcement actions, civil rights investigations, and judicial oversight applicable to Iowa's Department of Human Services and its successor agency, Iowa HHS. Together these records establish a documented pattern: when Iowa's internal correction mechanisms failed to produce conforming outcomes, external intervention — by the U.S. Department of Justice, federal courts, and consent decree monitoring — became necessary.

The escalation pattern documented here is directly continuous with the CFSR longitudinal record in Section 6. The same agency that failed three consecutive CFSR cycles (2003, 2011, 2019) is the same agency that required CRIPA enforcement action (2002, 2004), federal court monitoring (2023–2024), and class action consent decree oversight (1994–2003). Section 8 documents what happens when the procedural compliance framework of Section 6 reaches its limits.

IMPORTANT FRAMING NOTE: The DOJ investigations and federal court monitoring documented in this section concern Iowa DHS/HHS institutional care facilities — Glenwood and Woodward Resource Centers — rather than the child protective services system directly. They are included here for three reasons:

First, both institutional care and child protective services are administered by the same agency — Iowa DHS, now Iowa HHS. Systemic failures documented at one operational level of the agency are directly relevant to audit scope across all levels.

Second, the structural deficiencies identified across DOJ investigations — record-keeping

failures, inadequate individualized assessments, QA systems that do not detect their own failures, and improvement gains that reverse when oversight pressure is removed — are identical in character to the deficiencies documented in the CFSR longitudinal record.

Third, the active federal court monitoring as recently as May 2024 demonstrates that Iowa HHS remains subject to concurrent independent federal oversight mechanisms — none of which have produced durable systemic reform sufficient to close oversight.

Cross-reference: Section 6 (CFSR longitudinal record); Section 7 (Round 4 structural integrity); Section 13.2 (Systemic Failure Matrix); Section 19.3 (Oversight Gap Analysis).

8.2 ATTACHMENT A6 — DOJ CRIPA INVESTIGATIONS

8.2.1 2002 DOJ CRIPA Findings Letter

Woodward State Resource Center and Glenwood State Resource Center

Investigation initiated: March 22, 1999 | Findings letter issued: July 9, 2002

Authority: Civil Rights of Institutionalized Persons Act, 42 U.S.C. § 1997

Source: U.S. Department of Justice, Civil Rights Division

<https://www.justice.gov/crt/investigation-woodward-state-resource-center-and-glenwood-state-resource-center-iowa>

Facility visits: November 1999; April and May 2001.

Facility populations at time of review: Woodward (~280 residents); Glenwood (~385 residents). Both state-operated Iowa DHS facilities for individuals with developmental and mental disabilities.

DOCUMENTED FINDINGS — WOODWARD:

Restraint overuse: An average of 56 residents each spent approximately 33 hours per month in mechanical, manual, or behavioral restraints during the April 2000–March 2001 review period. Some individuals were restrained in excess of 300 hours per month. The DOJ found restraints were substituting for needed behavioral supports rather than functioning as emergency interventions — a constitutional violation under *Youngberg v. Romeo*, 457 U.S. 307 (1982).

Electronic tracking: Electronic tracking devices were used at Woodward for staff convenience rather than resident care — documented as a rights violation.

Habilitation: No evidence that behavioral causes of challenging behaviors were correctly assessed. Behavior support plans had no measurable impact on the behaviors they were designed to address.

DOCUMENTED FINDINGS — GLENWOOD:

General medical care: Lacked a medical director, peer review QA tool, pharmacy and therapeutics committee, and policies ensuring consistent care. Neurological resources insufficient for a medically fragile population.

DOCUMENTED FINDINGS — BOTH FACILITIES:

Psychiatric and psychological care: Inadequate assessments and diagnoses; unjustified use of psychotropic medications; poor monitoring of treatment outcomes; failure to obtain informed consent; inappropriate behavior plan development; non-integrated pharmacological and behavioral plans.

Community placement: Neither facility had established protocols ensuring adequate assessments to determine the most integrated setting appropriate for residents' needs — a direct violation of the ADA integration mandate under *Olmstead v. Zimring*, 527 U.S. 581 (1999).

Record-keeping: "Erroneous and incomplete record keeping plagued both facilities." (DOJ Findings Letter, July 9, 2002)

AUDIT RELEVANCE: The record-keeping failures documented at Woodward and Glenwood in 2002 are structurally identical to the data integrity failures documented two decades later in the KinderTrack independent analysis (Attachment A21, Section 10.4) and in official agency correspondence to Senator Ernst containing verifiable impossible case records (Attachment A43, Section 10.6). Iowa DHS's record-keeping failures are not a new finding — they are a documented institutional pattern spanning more than 20 years of agency operations.

Cross-reference: Section 6.6 Domain 3 (QA system — three-round ANI pattern); Section 10.4 (KinderTrack data integrity); Section 10.6 (Senator Ernst correspondence);

Section 13.2 (Systemic Failure Matrix — Domain H).

8.2.2 2020 DOJ Investigation — Glenwood State Resource Center

Source: Civil Rights Litigation Clearinghouse — <https://clearinghouse.net/doc/143683/>

The 2020 DOJ investigation reopened federal civil rights scrutiny of Glenwood Resource Center — the same facility investigated in 1999–2001 and subject to the 2004 consent decree. Its reopening establishes that the remedial measures imposed under *United States v. Iowa* (Section 8.4.2) did not produce durable systemic reform in the

approximately 16 years between the consent decree expiration (2010) and renewed federal scrutiny (2020).

AUDIT RELEVANCE: The recurrence of DOJ civil rights intervention at the same facility, under the same agency, within the same administrative period as the CFSR Round 3 failure (2018–2019) and the CFSR Round 3 PIP submission failure (2020), documents simultaneous multi-domain failure — child welfare outcomes, institutional care standards, and civil rights compliance deteriorating concurrently under Iowa HHS administration.

8.2.3 2021 DOJ Investigation — Glenwood and Woodward State Resource Centers

Source: Civil Rights Litigation Clearinghouse — <https://clearinghouse.net/doc/143684/>

The 2021 DOJ investigation expanded federal scrutiny to both Glenwood and Woodward, replicating the geographic scope of the original 1999–2001 CRIPA investigation. This expansion occurred:

- Two years after Iowa's CFSR Round 3 Final Report documented systemic regression across 5 of 7 systemic factors (Section 6.5)
- One year after Iowa's Round 3 PIP was submitted for the fourth time without a confirmed federal approval date (Section 6.5.4)
- Concurrently with Iowa's pursuit of a \$41.9M CCWIS contract lacking background check requirements for personnel accessing child welfare PHI (Section 18)

AUDIT RELEVANCE: The 2020–2021 DOJ investigations establish that Iowa's institutional care compliance failures are contemporaneous with — not historical antecedents to — the child welfare systemic failures documented throughout this petition. Both streams of federal concern were active simultaneously, within the same agency, during the same administrative period as the conduct subject to this audit petition.

8.3 ATTACHMENT A07 — FEDERAL COURT MONITORING REPORTS

Case: 4:22-cv-00398-RGE-SBJ | U.S. District Court for the Southern District of Iowa

Glenwood Resource Center — Consent Decree Monitoring

Docket: <https://www.courtlistener.com/docket/12009436/united-states-v-state-of-iowa/>

COMPLIANCE FRAMEWORK: The monitoring reports assess Glenwood Resource Center's compliance against the Consent Decree using the following thresholds:

- Substantial Compliance (SC): 80% or greater consistency
- Partial Compliance (PC): 50%–80% consistency

— Non-Compliance (NC): less than 50% consistency

8.3.1 October 18, 2023 Monitoring Report

Onsite dates: August 15–18, 2023 | Report date: October 18, 2023

Lead Monitor: James M. Bailey, MCD-CCC-SLP (Court Appointed)

Assistant Lead Monitors: Teri Towe, B.S.; Alan Harchik, Ph.D., BCBA-D

Source: gov.uscourts.iasd.81951.18.1

COMPLIANCE SUMMARY TABLE — OCTOBER 2023:

Section A (Research): Substantial Compliance — all paragraphs 41–47

Section B (Integrated Interdisciplinary Care): Partial Compliance on par. 49;

Non-Compliance on paragraphs 48, 50, 51, 52, 56, 57

Section C (Assessments and Clinical Care): Non-Compliance on the majority of paragraphs — 58, 62–67, 71–72, 76, 78, 80–86, 91, 94–99, 101, 103–106, 111, 113–115, 117, 119–120 all rated Non-Compliance

Section D (Restraints and Restrictive Procedures): Mixed — substantial compliance on some seclusion and other restrictive interventions provisions; non-compliance on paragraphs 124, 125, 132–134 (physical restraint protocols)

Section E (Engagement and Skill Acquisition): Partial Compliance on paragraphs 156, 157, 159; Non-Compliance on 155, 158, 160–163

Section F (Community Integration/Transition): Non-Compliance on 164, 165; Substantial Compliance only on 166

Section G (Organizational/Staffing): Non-Compliance on paragraphs 170–176

Section H (Health Care): Substantial Compliance on only 3 of approximately 35 paragraphs (179, 180, 190, 198); Non-Compliance on the majority including 181, 184–189, 191–196, 199, 203–205, 208

Section I (State Staff/Staffing): Partial Compliance — staffing levels fell short of the 1.8 relief factor required by the Consent Decree. As of the review, GRC had 216 filled Resident Treatment Workers and 31 Resident Treatment Supervisors, achieving only a 1.677 relief factor. "Staffing continued to be a barrier in meeting the needs of the individual." No policy existed for the staff complaint process.

Section J (Organizational Accountability): Mixed — no Resident Council in place; GRC policies had been updated but "lacked evidence of review and approval by HHS prior to implementation"

Section K (Quality Management): Partial Compliance on paragraphs 229–231; Substantial Compliance only on paragraph 233. The monitor found that "data

need to be added to the Quality Management program" including engagement, skill acquisition, choice/self-determination, staff capacity, and compliance with policies and procedures.

KEY MONITOR FINDINGS — OCTOBER 2023:

Person-centered planning: "An important component of person-centered planning missing from the ISP was the determination of what skills the individual wanted to learn and what supports were needed to participate fully in day and residential services in the most integrated setting." (Section B summary)

Skill acquisition plans: SAPs were in place for all reviewed individuals but were "not reflective of assessment results." 0% of SAPs were rated as practical, functional, and meaningful. Progress "could not be confirmed because SAP data were not reliable." (Section E)

Community transition: "For individuals who exhibited behaviors that were barriers to community transition, IDTs did not develop plans or strategies to minimize or overcome the barriers, and there was no evidence of a formal community integration plan." (Section E summary)

Staffing: Active recruitment was underway but the required 1.8 relief factor was not met. The closure process "further complicated the challenge for individuals living at GRC." (Section I)

Quality Management: The QM system lacked data in multiple required domains and "did not include evidence of HHS central office review of quality data." (Section K summary)

AUDIT RELEVANCE — OCTOBER 2023 REPORT:

The October 2023 monitoring report documents that Iowa HHS remained in Non-Compliance across the majority of Consent Decree provisions as of August 2023 — five years after the CFSR Round 3 onsite review (2018), three years after the Round 3 PIP final submission (2020), and during the same period Iowa HHS was seeking federal approval for a \$108.8M CCWIS investment (Section 18). The QA findings in Section K of the monitoring report — a QM system that does not collect data in required domains and lacks HHS central office review — directly parallel the QA system failures documented across three CFSR rounds (Section 6.6 Domain 3).

Cross-reference: Section 6.6 Domain 3 (QA system longitudinal failure);

Section 6.6 Domain 2 (workforce capacity pattern); Section 9 (current state operational reporting); Section 13.2 (Systemic Failure Matrix).

8.3.2 May 6, 2024 Monitoring Report

Onsite dates: March 18–21, 2024 | Draft report: April 14, 2024 | Final: May 6, 2024

Lead Monitor: James M. Bailey, MCD-CCC-SLP (Court Appointed)

Assistant Lead Monitor: Teri Towe, B.S.

Source: gov.uscourts.iasd.81951.41.1

Case filing: 4:22-cv-00398-RGE-SBJ Document 41-1 Filed 05/08/24

COMPLIANCE SUMMARY TABLE — MAY 2024:

Section A (Research): EXIT from active monitoring — two consecutive reviews in substantial compliance

Section B (Integrated Interdisciplinary Care): Non-Compliance on paragraphs 48–52, 56–57; Partial Compliance on 49 subsections

Section C (Assessments and Clinical Care): Continued high volume of Non-Compliance — paragraphs 97, 98, 99, 103, 104, 113, 114, 115, 117, 119, 120 all Non-Compliance; partial improvement from 2023 in some areas

Section D (Restraints): Substantial compliance expanded on most restraint provisions; Non-Compliance remains on paragraphs 124, 125, 134–140, 153

Section E (Engagement/Skill Acquisition): Non-Compliance on paragraphs 158, 160–163; Substantial Compliance on paragraph 156 only

Section F (Community Integration/Transition): Non-Compliance on 170–176; Substantial Compliance on 169 only — REGRESSION from 2023 on this section

Section G (Organizational/Staffing): Non-Compliance on 170–176

Section H (Health Care): Non-Compliance on majority — paragraphs 181, 184–189, 191–199, 200, 203, 204, 208 all Non-Compliance; limited improvement from 2023

Section I (State Staff): Partial Compliance — paragraphs 212, 214, 215

Section J (Organizational Accountability): Partial Compliance on 223, 225, 226; Non-Compliance on paragraph 224 only

Section K (Quality Management): Partial Compliance on 229–231; Substantial Compliance on 102, 232, 233

KEY MONITOR FINDINGS — MAY 2024:

Executive Summary admission: "Overall, improvement was noted for medical services, declines were seen in psychiatry and psychology, and limited movement

was seen with ISPs and overall client protections. The transition process continued to be the primary area of need as well as the supports/assessments leading into the plan." (Executive Summary, p.4)

Post-onsite state actions: The monitor acknowledged that since the March 2024 onsite review, the state had taken "considerable actions" including contracting a Transition Specialist (beginning April 3, 2024), contracting for dedicated psychiatry assistance, implementing warm hand-off procedures between center and community psychiatrists, and creating a "Strike Team" of clinicians for pre- and post-transition support. The monitor noted: "it should also be noted that many issues and subsequent recommendations had been noted since the baseline review." (Executive Summary, p.5)

This last observation is critical: the state implemented meaningful corrective actions in the weeks between the onsite review and the final report — not in the years preceding it. This is consistent with the pattern documented across the CFSR record in which Iowa DHS produces compliance outputs in response to imminent external review rather than as a function of sustainable internal quality improvement.

AUDIT RELEVANCE — MAY 2024 REPORT:

The May 2024 monitoring report establishes that as of March 2024 — the most recent federal court monitoring date documented in this petition — Iowa HHS remained in Non-Compliance across the majority of Consent Decree provisions governing health care (Section H), community integration (Section G), and clinical assessments (Section C). This is 22 years after the original 2002 DOJ findings letter, 20 years after the 2004 consent decree, and concurrent with the initiation of CFSR Round 4 under Executive Order 14359.

The coexistence of active federal court Non-Compliance findings and a new federal CFSR cycle designed with suspended PIP enforcement (Section 7) represents the convergence of two independent oversight failure streams within the same agency at the same point in time.

Cross-reference: Section 7 (Round 4 PIP suspension mechanism — EO 14359);

Section 6.6 Domain 5 (PIP accountability architecture — compliance outputs generated under review pressure, not sustained independently);

Section 19.3 (Oversight Gap Analysis).

8.4 ATTACHMENT A8 — HISTORICAL LITIGATION & REFORM EFFORTS

8.4.1 Conner v. Branstad

Case No. 4:86-cv-90871 | U.S. District Court for the Southern District of Iowa

Filed: December 8, 1986 | Closed: 2003

Source: Civil Rights Litigation Clearinghouse — <https://clearinghouse.net/case/446/>

CASE SUMMARY: Filed December 8, 1986 as a putative class action on behalf of residents of Glenwood and Woodward State Hospital-Schools. Plaintiffs represented by Iowa Protection and Advocacy Services, University of Iowa College of Law Clinical Program, the Legal Center for Special Education, and private counsel. Defendants: State of Iowa, Division of Mental Health/Mental Retardation, Iowa Department of Human Services.

CLAIMS: The lawsuit challenged Iowa's institutionally-based model of services for persons with developmental disabilities, alleging violations of:

- Title XIX of the Social Security Act
- Section 504 of the Rehabilitation Act
- Individuals with Disabilities Education Act (IDEA)
- Americans with Disabilities Act (ADA)
- First and Fourteenth Amendments via 42 U.S.C. § 1983

KEY PROCEDURAL HISTORY:

December 9, 1993: Court denied summary judgment in part; dismissed ADA and Rehabilitation Act claims. (Conner v. Branstad, 839 F.Supp. 1346 (S.D. Iowa 1993))

December 2, 1994: Consent Decree approved. Key provisions:

- Individualized treatment plans required for all class members
- Training and employment opportunities to be provided
- Class members notified of eligibility for community-based services
- State required to develop a five-year community support plan

March 12, 1996: Plan for Community Development filed — the five-year plan required under the Consent Decree. Six-month update filed May 1, 1996.

March–April 1997: Additional implementation correspondence filed.

2003: Plaintiffs' attorneys withdrew. Case closed — the same year Iowa received its first CFSR Final Report documenting systemic failure.

AUDIT RELEVANCE: Conner v. Branstad establishes that Iowa's institutional service model was subject to federal class action litigation from 1986 — seventeen years

before the first CFSR round. The Consent Decree's requirements for individualized treatment plans, community placement protocols, and five-year planning mirror the structural requirements that CFSR documented as deficient across three subsequent review cycles. The case closed in 2003 and was immediately followed by DOJ CRIPA enforcement for the same facilities and deficiency types — establishing a direct continuity of oversight concern across four decades.

Cross-reference: Section 6.2 (CFSR Round 1 — ICWA non-compliance, Woodward County); Section 8.2.1 (2002 DOJ findings at same facilities); Section 8.4.2 (United States v. Iowa — direct successor enforcement action).

8.4.2 United States v. Iowa

Case No. 4:04-cv-00636 | U.S. District Court for the Southern District of Iowa

Filed: November 18, 2004 | Closed: 2010

Source: Civil Rights Litigation Clearinghouse — <https://clearinghouse.net/case/9662/>

CASE SUMMARY: On November 18, 2004, the DOJ simultaneously filed a CRIPA complaint against Iowa and tendered a settlement agreement arising directly from the July 9, 2002 findings letter. The complaint alleged violations of Fourteenth Amendment due process rights and ADA rights at Woodward and Glenwood. Judge Ronald E. Longstaff approved the settlement on November 24, 2004 and retained jurisdiction to enforce it while administratively closing the case.

KEY DOCUMENTS:

— Complaint: November 18, 2004

— Settlement Agreement: November 24, 2004 (required remedial measures and periodic compliance reporting; authorized regular compliance reviews with facility inspections, staff/resident interviews, and full document access)

— Memorandum of Agreement (Woodward): April 1, 2009 (extended monitoring one additional year)

— Notice of Expiration: May 13, 2010 (both consent decrees expired; case closed)

TIMELINE INTERSECTION WITH CFSR RECORD:

November 2004: United States v. Iowa filed and settled — same year as Iowa's CFSR Round 1 PIP approval (August 2004). Two concurrent federal compliance mechanisms activated simultaneously for the same agency.

April 2009: Monitoring period extended at Woodward — same year CFSR Round 2

Statewide Assessment period begins.

May 2010: Consent decrees expired — same month as CFSR Round 2 onsite review preparation. Iowa's workforce had just absorbed the retirement of 638 IDHS staff effective June 24, 2010.

AUDIT RELEVANCE: The 2010 expiration of the United States v. Iowa consent decrees removed active federal judicial oversight from Iowa's institutional care system at precisely the moment Iowa was entering its second CFSR cycle — the cycle in which Iowa would fail all seven outcomes for the first time. The absence of judicial oversight following the 2010 expiration was followed, within ten years, by the return of DOJ civil rights scrutiny in 2020 and 2021, and by new federal court monitoring under a renewed consent decree as documented in Attachment A07.

This sequence — enforcement action, consent decree, compliance outputs, expiration, regression, renewed enforcement — is the same structural dynamic identified in the CFSR PIP record: gains produced under external pressure do not survive its withdrawal.

Cross-reference: Section 6.3.2 (Round 2 workforce collapse, June 2010);

Section 6.6 Domain 5 (PIP accountability — gains reversed when oversight removed);

Section 7 (Round 4 PIP suspension — same dynamic at federal administrative level);

Section 19.3 (Oversight Gap Analysis).

8.5 SECTION 8 SYNTHESIS — THE ESCALATION PATTERN

The documents comprising Section 8 establish a single recurring structural pattern across nearly 40 years of Iowa DHS/HHS oversight history:

PHASE 1 — Internal mechanisms fail to produce conforming outcomes

PHASE 2 — External oversight becomes necessary (DOJ, federal courts, litigation)

PHASE 3 — Enforcement produces procedural compliance outputs (consent decrees, PIPs, monitoring reports, settlement agreements)

PHASE 4 — External oversight pressure is withdrawn or expires

PHASE 5 — Documented regression follows

PHASE 6 — External oversight becomes necessary again

This cycle has completed multiple iterations:

Conner v. Branstad (1986–2003) → United States v. Iowa (2004–2010)

CFSR Round 1 PIP approved (August 2004) → Round 2 all-outcomes failure (2010)

United States v. Iowa expired (May 2010) → DOJ reinvestigation (2020–2021)

CFSR Round 2 PIP approved (October 2011) → Round 3 systemic regression (2018)
Round 3 PIP submitted without approval (May 2020) → Active court monitoring
finding majority Non-Compliance (2023–2024)

The petition to audit under Iowa Code § 11 is a request for an independent examination of whether this cycle can be interrupted — not by adding another external compliance mechanism, but by determining whether Iowa HHS is structurally capable of producing the outcomes it reports, and whether the oversight mechanisms designed to verify those outcomes are themselves producing reliable determinations.

Section 8 establishes that this question has been raised, in various forms, for four decades. It has not yet received a durable answer.

Source attachments for this section:

A6 — DOJ CRIPA Investigations (2002, 2020, 2021) — evidentiary excerpts

A07 — Federal Court Monitoring Reports (October 18, 2023; May 6, 2024)

A8 — Conner v. Branstad and United States v. Iowa litigation records

8.X DOMAIN & DEFICIENCY MAPPING — SECTION 8

Domain D — Judicial Interface & Legal Oversight (primary):

F11 — Interagency Coordination Failure: Four decades of federal court involvement — CRIPA, DOJ, monitoring — without producing durable systemic correction establishes a pattern of institutional non-response to external oversight intervention.

F12 — Supervisory Review & Escalation Failure: Court monitoring reports (2023, 2024) document continued deficiencies in supervision and escalation despite active federal judicial oversight.

Domain G — Federal Oversight History & Corrective Action Response:

F5 — Corrective Action Failure: Conner v. Branstad consent decree and subsequent federal court monitoring produced documented short-term improvements that did not survive budget reversals — the same pattern documented in CFSR PIP analysis in Section 6.

Domain F — Workforce Capacity & Internal Accountability:

F9 — Workforce Capacity & Caseload Crisis: Federal court monitoring reports cite caseload and staffing deficiencies as ongoing concerns during active court oversight — consistent with the longitudinal

workforce record in Section 6.

8.X AUDIT QUESTIONS GENERATED — SECTION 8

AUDIT QUESTION 8-A: The 2023 and 2024 federal court monitoring reports document continued deficiencies during active judicial oversight. What specific findings in those reports have been addressed by Iowa DHHS, and what is the current compliance status with each outstanding monitoring requirement?

AUDIT QUESTION 8-B: The Conner v. Branstad consent decree produced documented improvements that were reversed following budget decisions. What structural safeguards, if any, prevent a similar reversal of improvements achieved under the current monitoring framework?

AUDIT QUESTION 8-C: Given four decades of federal judicial involvement in Iowa child welfare oversight, what determination has Iowa DHHS made about why court-supervised improvements have not produced durable systemic change, and what evidence supports that determination?

AI assistance disclosure: See Section 5.8.
to the attachments identified above.

End of Section 8 — Version 1.0 — June 5, 2026

Audit Petition Packet v2.0 (5/2026)

AI assistance disclosure: See Section 5.8.

AI assistance was used materially in the preparation of this document. That assistance does not constitute independent verification of any factual claim.